

BADIRA — Preeclampsia Risk-Factor Screening

Research notes — How a woman is flagged as high-risk before anything goes wrong

1. What This Screening Route Is

Before preeclampsia develops clinicians can flag women who have characteristics linked to a higher risk of developing it. The SMFM/ACOG checklist route is based mainly on maternal medical obstetric and family history plus basic maternal characteristics.

Key point: this is risk assessment not diagnosis. It asks who is more likely to develop preeclampsia later.

2. High-Risk Factors

High-risk factor	Meaning
Previous preeclampsia	Preeclampsia in a previous pregnancy
Multifetal gestation	Current pregnancy with twins or more
Chronic hypertension	Pre-existing chronic high blood pressure
Type 1 or type 2 diabetes	Pregestational diabetes
Renal disease	Pre-existing kidney disease
Autoimmune disease	For example systemic lupus erythematosus or antiphospholipid syndrome

Decision rule: If a woman has at least one high-risk factor, ACOG/SMFM recommend low-dose aspirin to help prevent preeclampsia, when appropriate.

3. Moderate-Risk Factors

Moderate-risk factor	Meaning
Nulliparity	No previous birth
Obesity	Prepregnancy BMI ≥ 30 kg/m ²
Family history	Mother or sister had preeclampsia
Low socioeconomic status	A social-risk factor in the 2020 checklist
Maternal age ≥ 35 years	Age 35 years or older
Prior low-birthweight / SGA history	Relevant previous low-birthweight or small-for-gestational-age history
Previous adverse pregnancy outcome	Relevant adverse outcome in a previous pregnancy
Interpregnancy interval >10 years	More than 10 years since the previous pregnancy
Race factor	Included as a moderate-risk factor in the U.S.-based checklist, reflecting social and healthcare disparities rather than biological differences

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Decision rule: with more than one moderate-risk factor low-dose aspirin should be considered/recommended according to the applicable ACOG/SMFM guidance.

4. What Happens After a Woman Is Flagged?

ACOG/SMFM recommend low-dose aspirin 81 mg/day for women who meet the risk-factor criteria. It is started between 12 and 28 weeks of gestation and ideally before 16 weeks and continued until delivery.

Important: being high-risk does not mean the woman has preeclampsia or will definitely develop it.

5. What Does This Route Require?

The checklist uses information that can be collected early in prenatal care: previous pregnancy history, current pregnancy information, medical history, family history, age, and prepregnancy BMI.

No specialized preeclampsia biomarker test or uterine artery Doppler is required for this checklist route. Those measurements belong to the FMF screening pathway in the next task.

6. Main Limitation

The checklist identifies increased risk but cannot tell with certainty who will develop preeclampsia. Its result also depends on having a complete and accurate history. A woman without recognized checklist risk factors can still develop the disease.

7. Why This Matters for BADIRA

Many checklist inputs are information BADIRA could organize early in pregnancy such as previous preeclampsia, chronic conditions, previous pregnancy outcomes, family history, age, and pregnancy history.

8. Clinical Path in One View

1. Initial prenatal visit
2. Collect medical obstetric and family history + maternal characteristics
3. Check high-risk and moderate-risk factors
4. ≥ 1 high-risk factor $\rightarrow$ low-dose aspirin recommended when appropriate
5. > 1 moderate-risk factor $\rightarrow$ low-dose aspirin considered/recommended according to the applicable guidance
6. Continue antenatal surveillance
7. Risk screening $\neq$ diagnosis of preeclampsia

9. The Updated 2026 SMFM Checklist

SMFM published an updated checklist in 2026 that replaces the 2020 statement. The 2026 SMFM checklist replaces the 2020 version. The main updates are:

- IVF is added as a moderate-risk factor.
- Sociodemographic factors are updated, emphasizing that increased risk associated with race reflects social inequities and racism rather than biological differences.

10. Sources

1. SMFM Special Statement: Checklists for Preeclampsia Risk-Factor Screening to Guide Recommendations for Prophylactic Low-Dose Aspirin (2020)

2. ACOG/SMFM Practice Advisory: Low-Dose Aspirin Use for the Prevention of Preeclampsia and Related Morbidity and Mortality (2021)

[ACOG — 2021 Practice Advisory](#)

[SMFM — 2021 Practice Advisory](#)

3. SMFM Special Statement: Updated Checklists for Preeclampsia Risk-Factor Screening to Guide Recommendations for Prophylactic Low-Dose Aspirin (2026)

[SMFM — 2026 Updated Checklist](#)